Chronic Kidney Disease (CKD): understanding your kidney reports and care

> *A practical guide to understanding kidney reports, blood pressure and anaemia in chronic kidney disease. It explains the main points in everyday language. Your own treatment plan should always be discussed with your clinician.*

1. Definition and chronicity

CKD is an abnormality of kidney structure or function, present for >3 months, with implications for health. A single raised creatinine or one low eGFR does not by itself establish CKD. Distinguish chronic disease from AKI or transient changes.

2. Classify by CGA

Record Cause, GFR category (G1–G5) and Albuminuria category (A1–A3). GFR and albuminuria together stratify kidney and cardiovascular risk. G1 or G2 eGFR alone does not establish CKD without another marker of kidney damage.

3. GFR and albuminuria categories

G1 ≥90; G2 60–89; G3a 45–59; G3b 30–44; G4 15–29; G5 <15 mL/min/1.73 m^2. Urine ACR: A1 <30 mg/g; A2 30–300 mg/g; A3 >300 mg/g. Prefer a spot urine ACR (often first-morning); confirm abnormality in context. Risk rises as eGFR falls and albuminuria increases.

4. Measure BP correctly before applying a target

KDIGO 2021 recommends standardized office BP rather than routine office measurement. Use correct cuff size, adequate rest and standardized positioning/ technique. HBPM or ABPM can complement standardized office readings. Do not apply the SBP <120 target to non-standardized readings.

5. BP target and RAS blockade

For most adults with CKD not receiving dialysis, KDIGO 2021 suggests standardized-office SBP <120 mmHg when tolerated (2B). Consider less intensive therapy with symptomatic postural hypotension or very limited life expectancy. Start ACEi/ARB for high BP with CKD and A3 without diabetes (recommended); A2 without diabetes (suggested); or A2/A3 with diabetes (recommended). Use the highest approved tolerated dose when indicated.

6. ACEi/ARB monitoring and safety

Check BP, serum creatinine and potassium within 2–4 weeks after starting or increasing ACEi/ARB, adjusted to eGFR and potassium. Investigate a creatinine rise >30% within 4 weeks. Consider dose reduction/cessation for symptomatic hypotension or uncontrolled hyperkalemia despite treatment. Avoid combining ACEi, ARB and direct renin inhibitor. Where possible, manage hyperkalemia rather than reflexively stopping RAS inhibition.

7. Lifestyle: sodium and activity

For high BP and CKD, KDIGO 2021 suggests sodium <2 g/day (equivalent to salt <5 g/day). This may not suit sodium-wasting nephropathy. Potassium-rich salt substitutes or a DASH-type diet may be unsafe in advanced CKD or impaired potassium excretion. Suggest moderate activity for at least 150 minutes/week, or as compatible with cardiovascular and physical tolerance.

8. Anaemia screening — KDIGO 2026

Test at referral, regularly during follow-up and when symptoms raise suspicion. Initial panel: CBC, reticulocytes (reticulocyte production index), ferritin and TSAT. Minimum suggested frequency: CKD G3 annually; G4 twice yearly; G5 or dialysis every 3 months. Test sooner when clinically indicated.

9. Iron treatment thresholds — haemodialysis

In anaemic CKD G5 haemodialysis, consider iron when ferritin is at or below 500 ng/mL and TSAT is at or below 30% (2D). IV iron is suggested over oral iron when initiating therapy.

१०. Dialysis नसल्यास किंवा peritoneal dialysis असल्यास

Dialysis नसलेल्या CKD रुग्णांत किंवा peritoneal dialysis घेत असताना ferritin 100 पेक्षा कमी आणि TSAT 40% पेक्षा कमी, किंवा ferritin 100 ते 300 दरम्यान आणि TSAT 25% पेक्षा कमी असेल तर iron देण्याचा विचार होतो. Iron गोळ्यांनी द्यायचे की शिरेतून, हे त्रासाची तीव्रता, औषध सहन होते का, उपलब्धता आणि खर्च पाहून ठरवले जाते.

११. Iron कधी थांबवण्याचा विचार करतात?

Ferritin 700 पेक्षा जास्त किंवा TSAT 40% किंवा अधिक असल्यास नेहमीचे iron उपचार थांबवण्याचा विचार केला जातो. हे मार्गदर्शक तत्त्वांतील निकष आहेत. एका रिपोर्टवरून स्वतःहून iron सुरू किंवा बंद करू नका.

१२. ॲनिमियासाठी पुढील उपचार

Iron घेत असल्यास Hb, ferritin आणि TSAT पुन्हा तपासावे लागतात. गोळ्यांनी फायदा होत नसेल किंवा त्या सहन होत नसतील तर डॉक्टर शिरेतून iron देण्याचा विचार करू शकतात. पुढील उपचार ठरवण्यापूर्वी रक्तस्राव, iron कमी असणे, B12/folate ची कमतरता किंवा इतर कारणे तपासली जातात. काही रुग्णांना ESA इंजेक्शनची गरज लागू शकते. Hb किती आहे एवढेच न पाहता लक्षणे, इतर आजार आणि रक्त देण्याची गरज यांचा एकत्र विचार केला जातो. हे उपचार डॉक्टरांच्या देखरेखीखालीच घ्यावेत.

१३. रोजच्या उपचारात काळजी घ्या

किडनीवर परिणाम करू शकणारी painkiller औषधे (विशेषतः NSAIDs) डॉक्टरांचा सल्ला न घेता घेऊ नका. काही औषधांची मात्रा किडनीच्या कामानुसार बदलावी लागते. BP, creatinine/eGFR, potassium आणि लघवीतील प्रथिने यांचे रिपोर्ट वेळोवेळी पाहा. लघवी अचानक खूप कमी होणे, सूज किंवा दम झपाट्याने वाढणे, छातीत दुखणे, बेशुद्ध पडणे किंवा potassium खूप वाढल्याचा संशय असल्यास तातडीने वैद्यकीय मदत घ्या.

For a life-threatening emergency in India, call 112. Elsewhere use your local emergency number.

१४. पुढच्या भेटीपूर्वी ही यादी पाहा

☐ किडनीचा त्रास ३ महिन्यांपेक्षा जास्त काळ आहे का हे स्पष्ट आहे? ☐ eGFR आणि urine ACR चे रिपोर्ट आहेत का? ☐ BP योग्य पद्धतीने मोजला आहे का? ☐ औषधे आणि त्यांची मात्रा डॉक्टरांनी तपासली आहे का? ☐ Hb, ferritin आणि TSAT तपासण्यांची तारीख ठरली आहे का? ☐ किडनी तज्ज्ञांचा सल्ला कधी घ्यायचा हे ठरले आहे का?

Questions you may have

Does one eGFR of 58 establish CKD?

No. Establish chronicity, repeat eGFR where appropriate, assess urine ACR and other markers of kidney damage.

Is SBP <120 mandatory for every person with CKD?

No. The KDIGO 2021 target applies to adults not receiving dialysis, using standardized office measurement, when tolerated. Do not apply it to non-standardized readings.

Should ACEi/ARB be stopped immediately if creatinine rises?

Not automatically. Check within 2–4 weeks after initiation or dose increase; investigate a rise >30% within 4 weeks in clinical context.

Is the KDIGO 2026 anaemia guideline a complete CKD guideline?

No. It focuses on diagnosis and management of anaemia in CKD, including iron, ESA/HIF-PHI and transfusion.

What to remember

Use both eGFR and urine ACR to understand CKD. Set blood pressure and treatment goals according to the person, not just one report. Keep follow-up tests on time and never change medicines on your own.

Sources and further reading

KDIGO 2024 — CKD Evaluation and Management guideline
https://kdigo.org/guidelines/ckd-evaluation-and-management/

KDIGO 2021 — Blood Pressure in CKD guideline
https://kdigo.org/guidelines/blood-pressure-in-ckd/

KDIGO 2026 — Anemia in CKD guideline
https://kdigo.org/guidelines/anemia-in-ckd/

Government of India — Emergency Response Support System 112
https://112.gov.in/about

Educational information supports understanding and does not replace individual medical care. Follow your clinician’s plan. Do not change medicines on your own. Sources checked: 24 October 2026.

Website: https://home-visiting-icu-doctor.vercel.app/#wikiSection
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